

HOW TO OPEN A BRAIN INJURY RESIDENTIAL CARE FACILITY IN OHIO

An Ohio-licensed Residential Care Facility (RCF) serving adults with brain injury — residential care, personal services, medication administration, and support toward greater independence.

A step-by-step startup and operations guide

Licensing (ODH) · Staff & training · Funding · The program · Accreditation

Regulatory basis: Ohio Administrative Code Chapter 3701-16 (Residential Care Facilities) and Ohio Revised Code Chapter 3721; licensed by the Ohio Department of Health (ODH). Funding: the Medicaid Assisted Living Waiver, MyCare Ohio, and private pay.

How to Use This Guide

This guide walks you through opening and operating a Residential Care Facility (RCF) in Ohio that serves adults with brain injury. An RCF is Ohio's assisted-living license — licensed by the Ohio Department of Health (ODH) under Ohio Administrative Code Chapter 3701-16 and Ohio Revised Code Chapter 3721 — and, unlike Ohio's smaller 'adult care facility' license, an RCF may administer medications and provide limited nursing, which is why it fits a brain-injury population. There is one licensing path (get the home licensed as an RCF) and a funding picture built around the Medicaid Assisted Living Waiver, MyCare Ohio, and private pay.

Important — verify before you act

This guide is an educational resource, not legal, financial, tax, or billing advice. Requirements, forms, fees, and rates change. Confirm every step with ODH and, for funding, with the Ohio Department of Aging, your regional PASSPORT Administrative Agency, and the resident's payer before you act. An RCF provides personal care, supervision, medication administration, and only the limited part-time skilled nursing O.R.C. 3721.011 allows; a resident who needs more must be served at a higher level of care.

The big picture, in one paragraph

Here is the whole business in a nutshell. You license a home with the State (ODH) as a Residential Care Facility built for adults with brain injury. You staff it with screened, trained people who can administer medications, and run it on written policies. You fill it through relationships with rehabilitation hospitals, trauma centers, and waiver case managers. And you fund it through a blend of the Assisted Living Waiver (Medicaid), MyCare Ohio, and private pay — with private pay as a deliberate base, because the waiver has limited slots. One caution shapes everything: the Ohio Home Care Waiver, which many assume covers brain injury, does not pay licensed facilities — so the RCF-plus-Assisted-Living-Waiver path is the one that works. License it, staff it, fill it, fund it.

The path at a glance — step by step

1. Set up the business — form your Ohio entity, secure the home, and confirm local zoning allows a residential care facility at the address.
2. Get the home ready — meet the O.A.C. 3701-16 physical-plant standards, obtain a certificate of occupancy (use group R-4, I-1, or I-2), and pass fire-marshall and heating inspections.
3. Screen and train staff — criminal records checks (O.R.C. 3721.121), required training, and qualified medication-administration personnel.
4. Write your policies — your 3701-16 policy manual and the forms it references (required for the application).
5. Apply to ODH — submit the RCF license application at least 60 days before opening, with the CPA solvency statement, ownership, floor plan, occupancy, and inspection reports, and the fee.
6. Pass the licensure inspection — ODH issues your license once you demonstrate compliance (an expedited inspection is available for an added fee).
7. Certify as an Assisted Living Waiver provider — through the Ohio Department of Aging, and enroll as an Ohio Medicaid provider.
8. Line up funding and referrals — connect with your PASSPORT Administrative Agency, MyCare plans, and rehab-hospital discharge planners.
9. Admit and document — welcome residents, deliver services, and document to stay survey-ready.

Part 1 — What This Business Is

An Ohio Residential Care Facility provides accommodations, supervision, personal care, and meals to adults, and — critically for brain injury — may administer medications, supervise special diets, apply dressings, and provide skilled nursing on a part-time, intermittent basis (up to 120 days in any 12-month period, or longer where a physician determines the need is routine). That medication and nursing authority is what separates an RCF from Ohio's smaller adult care facilities, which are prohibited from administering medications.

License: Residential Care Facility, issued by the Ohio Department of Health under O.A.C. Chapter 3701-16 and O.R.C. Chapter 3721.

Why brain injury is its own niche — and why the RCF license fits

Many brain-injury residents are younger adults — often 21 to 64 — who were independent before a crash, fall, assault, stroke, or overdose. They may walk and talk normally but struggle with memory, initiation, judgment, impulse control, and fatigue, and they frequently take anticonvulsants and other time-sensitive medications that must be administered accurately and on schedule. A home that can administer those medications, provide structure and supervision, and support skill-building is genuinely hard to find — which is why hospitals, families, and case managers are looking for you, and why the RCF (not the adult-care-facility) license is the right vehicle.

What you are actually selling is trust and specialization: a safe, structured home where a person with a brain injury can rebuild daily-living skills and, where possible, move toward greater independence — with medications handled reliably by trained staff. That specialization lets you stand apart from ordinary residential care facilities and adult family homes, build referral relationships with rehabilitation hospitals, and, with accreditation, strengthen your payer relationships. The demand is real and under-served: Ohio's Assisted Living Waiver has open capacity precisely because too few providers have stepped up, so a well-run specialized brain-injury home is entering a market that needs it.

Part 2 — Who Regulates You

- **Ohio Department of Health (ODH)** — licenses your RCF, sets the 3701-16 standards, and conducts the licensure and complaint surveys through its Bureau of Long-Term Care / Office of Health Assurance and Licensing.
- **Ohio Department of Aging (ODA) & ODM** — administer the Assisted Living Waiver that funds services in your RCF; ODA certifies waiver providers, and your regional PASSPORT Administrative Agency (PAA) handles enrollment and case management.
- **MyCare Ohio plans** — managed-care plans that arrange waiver services for dual-eligible residents in the 29 MyCare counties.
- **Long-Term Care Ombudsman & Adult Protective Services** — resident advocacy and the investigation of abuse, neglect, and exploitation.

Keep the two sides straight, because it saves you weeks of being bounced around. ODH is your licensing regulator — it decides whether you can open, inspects you, and can cite or fine you. ODA/ODM and the PAAs are your payer side — being licensed by ODH does not make you a Medicaid provider, and being a Medicaid provider does not license your home; they are separate applications through separate offices. MyCare plans are the managed-care route to that same waiver funding for dual-eligible residents in the MyCare counties. And the Ombudsman and Adult Protective Services are oversight bodies you'll interact with around resident rights and safety — the Ombudsman advocates for residents and helps resolve complaints, while ODH and the county investigate abuse and neglect. Knowing which office owns which question is why the contact list near the end of this guide is organized by what you need, not by agency name.

Part 3 — Business and Insurance Setup

- Form your Ohio entity (usually an LLC or corporation) with the Ohio Secretary of State, get an EIN, and open a business bank account — keep business and personal funds strictly separate.
- Confirm in writing that local zoning allows a residential care facility at the exact address before you sign a lease or buy.
- Secure liability, professional, property, and workers' compensation insurance appropriate to a licensed residential care facility — tell the insurer plainly that you serve adults with brain injury.
- Identify all owners of five per cent or more (required on the license application) and prepare the CPA financial-solvency statement (three-month reserve).

- Obtain a National Provider Identifier (NPI) and plan Ohio Medicaid enrollment for waiver billing.

What each of these really means

The entity is your legal shield: form an LLC or corporation so the business — not you personally — holds the license, signs the lease, and carries the liability, and keep the business's money entirely separate from your own, because commingling is one of the fastest ways to lose that protection. The CPA financial-solvency statement is not a formality — Ohio requires proof of at least three months of operating reserve at licensure, so line up your financing before you apply.

Zoning is the step first-time operators most often underestimate. Before you sign a lease or buy, get written confirmation that a residential care facility is an allowed use at the specific address — rules differ by city and suburb, and metro Columbus in particular is a patchwork of separate jurisdictions. The good news: homes for people with disabilities have protections under the federal Fair Housing Act, so a blanket 'no group homes' answer is often not the final word — but settle it before money is at risk.

Insurance is not one policy: plan for general liability, professional (or 'abuse and molestation') liability, property coverage, workers' compensation once you have employees, and commercial auto if you transport residents. Tell the insurer plainly that you serve adults with brain injury in a licensed residential care facility that administers medications — underpricing the risk by describing it as ordinary housing leaves you uncovered when it matters. The NPI is free and can be obtained early, before the home opens.

Part 4 — The Home: Physical Plant and Fire Safety

Your building must meet the O.A.C. 3701-16 physical-plant standards and hold a certificate of occupancy in use group R-4, I-1, or I-2. Before your licensure inspection, confirm:

- ☐ A certificate of occupancy in the applicable use group (R-4, I-1, or I-2).
- ☐ A current state fire marshal (or approved local fire department) inspection and the required fire-protection and life-safety systems.
- ☐ A current heating inspection.
- ☐ Resident bedrooms, sufficient bathrooms, and common areas that are safe, sanitary, and in good repair.
- ☐ An accessible, low-clutter environment that reduces confusion and falls, with secure storage of anything hazardous.
- ☐ Individualized evacuation planning for residents whose brain injury affects mobility or cognition.

What the inspection actually checks — and where homes lose points

The physical-plant inspection is concrete and winnable if you prepare. Surveyors check that the certificate of occupancy is in the right use group, that fire-marshal and heating inspections are current, that bedrooms and bathrooms meet the standards for your resident count, that hot-water temperature is safe, that exits are clear and operable, that medications and hazardous chemicals are stored securely, and that the home is clean and in good repair.

The deficiencies that most often delay a first license are small and avoidable: expired fire-extinguisher tags, a water heater set too hot, no documented fire drills, blocked or improperly locked exits, an incomplete first-aid kit, and gaps in required postings. Walk your home with a surveyor's eyes a week ahead, fix the small things, and photograph your fire-safety equipment and drills.

Design for brain injury goes beyond the code minimum. A calm, low-clutter, clearly organized environment reduces confusion and falls; labeling, cueing, and consistent placement support residents with memory and organization deficits; safe water temperature and secure storage of medications and hazards protect residents with impaired safety awareness; and good lighting and clear sightlines help staff supervise. Think through elopement risk at the design stage — door and exit awareness, monitoring, and secure storage of anything harmful — while keeping the

home homelike rather than institutional, because over-restriction erodes the independence your program is meant to build.

Part 5 — Administrator, Staff, and Required Training

- **Administrator:** the RCF operates under an administrator responsible for daily operation, with a competent person designated to cover any absence.
- **Criminal records checks:** every person in a resident-contact position obtains the criminal records check required by O.R.C. 3721.121 before working with residents, and is re-checked as required.
- **Training:** staff complete the O.A.C. 3701-16-06 training — residents' rights, population needs, abuse reporting, emergencies, infection control — plus brain-injury, seizure-response, and de-escalation content, with competency validated and documented.
- **Medication personnel:** medications are administered only by authorized qualified persons under O.A.C. 3701-16-09 and O.R.C. 3721.011(B); staff who only assist with self-administration do so within the limits of paragraph (F) after required training.

The training your staff actually need

Ohio is specific about training, and surveyors check it. Staff complete the O.A.C. 3701-16-06 training — residents' rights, the needs and characteristics of the resident population, abuse and neglect recognition and reporting, emergency and fire procedures, and infection control — and staff who assist with medication self-administration complete the specific training paragraph (E) requires before they assist, with competency validated. Build a training calendar and a signed record for each person; 'we trained them' without documentation counts as not trained on survey day.

For a brain-injury home, go beyond the minimum with content your competitors skip: how injury to different parts of the brain affects memory, attention, and behavior; cueing and compensatory strategies; recognizing and de-escalating agitation; seizure recognition and response; fall prevention; and each resident's individual support strategies so care is consistent across shifts. This training is also a selling point — payers and accreditors want to know your staff are prepared for this population.

Medication administration — your clinical edge

The RCF's authority to administer medications is the reason it fits brain injury, so build the staffing to deliver it. Medications are administered by a licensed nurse or other authorized, qualified staff, or through a contracted home health or hospice provider; unlicensed staff who merely assist a self-administering resident are limited to reminding, watching, and retrieving and handing over medication from locked storage after verifying the label and the resident's identity. Because many brain-injury residents cannot safely self-administer — impaired memory or judgment makes missed or doubled doses a real danger — plan for administration by authorized personnel, and keep the line between 'administration' and 'assistance' clear, resident by resident.

Supervision matched to real needs

Brain injury drives your staffing model in two directions at once. Residents need closer supervision — for elopement and safety risk, impulsivity, fall risk, and behavioral support — and they need consistency, the same trained staff who know each resident's cues and strategies. So set supervision and check-in levels from each resident's assessment rather than a one-size rule, provide awake overnight coverage where residents' needs or the census require it, and remember that a resident-call system is only one layer: some residents cannot reliably use one or recognize when they need help, so proactive monitoring is part of the job. Match supervision to assessed need, and expand a resident's independence as they demonstrate readiness rather than defaulting to blanket restriction.

Part 6 — Get Your RCF License from ODH

1. Download the RCF license application from ODH and submit it at least 60 days before your intended opening date.
2. Include the required documents: the CPA financial-solvency statement (three-month reserve), ownership disclosures (5%+), floor plan, certificate of occupancy, and fire-marshall and heating inspection reports.
3. **Pay the license fee.** An expedited initial inspection is available for an additional \$2,250, with the inspection scheduled within 10 business days (O.A.C. 3701-16-03.1). Confirm the current base fee with ODH.
4. Have your 3701-16 policies and procedures complete — the application requires them.
5. Pass the licensure inspection; ODH issues your license once you demonstrate compliance. Until licensed, do not accept more than two residents who need personal care or medication administration.

The documents you'll assemble

Give yourself time to gather these before the 60-day clock matters — several take outside parties to produce:

- ☐ The completed ODH application form and license fee.
- ☐ The CPA-signed financial-solvency statement showing at least three months of operating reserve.
- ☐ Ownership disclosures for every owner of five per cent or more.
- ☐ A floor plan of the home.
- ☐ The certificate of occupancy (use group R-4, I-1, or I-2).
- ☐ Current fire-marshall and heating inspection reports.
- ☐ Your complete 3701-16 policies and procedures, and the forms they reference.

Where your manual and forms fit. Your Policy & Procedure Manual and Forms Packet are built to the 3701-16 requirements — have them ready before you apply.

What the inspection looks at

Expect the surveyor to tour the home, review your policies and personnel files (qualifications, criminal records checks, training), check the physical plant and fire safety, and confirm your admission, assessment, medication, and resident-record systems match your policies. The inspection is not a pop quiz — it is a check that the home you described on paper actually exists and runs the way your manual promises. If you have real policies, screened and trained staff, a safe building, and organized records, you are in good shape.

If deficiencies are found, you are typically given the chance to submit a plan of correction rather than being denied outright — a written statement of what you will fix, how, and by when. Correct the root cause, not just the single instance, document the fix, and treat it as genuine improvement; homes that do clear it quickly. Plan for months, not weeks, from application to license, and remember you must also complete Assisted Living Waiver certification and Medicaid enrollment before waiver funding can flow — so budget the runway for an empty, licensed home for a while.

Part 7 — How You Get Paid — Funding the Home

Here is the honest funding picture. Ohio has no single published brain-injury per-diem for residential care; you build funding from these sources:

Source	What it pays / how	The reality
Assisted Living Waiver (ODA / ODM)	Pays for services (not room and board) in an ODH-licensed Residential Care Facility, for adults 21+ at a nursing-facility level of care, on a two-tier service payment (Basic and Memory Care) set by the Ohio Department of Aging. You license the RCF with ODH,	Room and board is the resident's responsibility (\$944/month in 2026). Income limit 300% of the Federal Benefit Rate (\$2,982/month in 2026); countable assets ≤ \$2,000. There is a

Source	What it pays / how	The reality
	then certify as an Assisted Living Waiver provider through ODA and enroll as a Medicaid provider.	waitlist — but largely because providers are scarce (Medicaid pays below private pay), which is an opening for a specialized home.
MyCare Ohio	Managed-care program for people with both Medicare and Medicaid in 29 counties — including Franklin (Columbus) and Hamilton (Cincinnati). Waiver services are arranged through the member's MyCare plan (Aetna, Buckeye, CareSource, Molina, or UnitedHealthcare).	The member requests the waiver through their MyCare plan care manager.
Private pay	Resident or family pays for services and room and board under the resident agreement.	Set by your rate schedule.

The catch that trips people up

The Ohio Home Care Waiver — the waiver most associated with younger adults with disabilities — explicitly excludes licensed residential facilities (it funds people in their own homes). It will NOT pay your RCF. For a residential brain-injury home, the Medicaid path is the Assisted Living Waiver (plus MyCare Ohio), which only a licensed RCF can bill — that is why the license choice matters.

Becoming an Assisted Living Waiver provider

The sequence is: license the home with ODH as an RCF, then apply to the Ohio Department of Aging for Assisted Living Waiver provider certification, then enroll as an Ohio Medicaid provider (you'll be assigned a Medicaid provider number) and sign a Provider Agreement with your regional PASSPORT Administrative Agency. The waiver pays a two-tier service rate set by ODA; room and board is the resident's responsibility. Because providers have been slow to join (Medicaid pays below private pay), a well-run specialized home is genuinely in demand.

Go in clear-eyed on the mechanics. The waiver pays for services only — the resident pays room and board (\$944/month in 2026, tied to the Social Security benefit rate) — so your resident agreement and your rate schedule have to handle that split cleanly, and the resident must be financially eligible (income within 300% of the Federal Benefit Rate, \$2,982/month in 2026, and countable assets no more than \$2,000) and assessed at a nursing-facility level of care. Enrollment is coordinated through the PAA, and the enrollment date is the day the resident moves in. Confirm the current tier rates with ODA before you build a budget — there is no single public per-diem to quote, and the tier a resident falls into depends on their assessed needs.

Verify each resident's funding before admission

The Assisted Living Waiver has limited slots and financial and level-of-care requirements, MyCare runs through the member's plan, and the Home Care Waiver won't pay a facility at all. Confirm each resident's specific coverage — and what it will and will not pay — before you admit.

Pricing your private-pay rate

Private pay is part of your core model, not a fallback, so price it deliberately from your real costs — the home, staffing (your largest cost, and higher for brain injury because of the supervision and medication administration required), food, utilities, insurance, training, and a margin — and put the rate, what it includes, and your refund and rate-change terms clearly in the resident agreement.

The other money — and one alternate path

- **Veterans:** residents who are veterans may have VA benefits; VA medical centers are also a referral source.

- **Auto & workers' comp:** because many brain injuries are accident-related, a resident may have auto-insurance or workers'-compensation coverage for care — always ask.
- **A different path for early-onset injury:** if a resident's brain injury occurred before age 22, it may count as a developmental disability, which opens the DODD waivers (Individual Options, Level One) through the County Board of Developmental Disabilities — a separate system from the RCF/Assisted Living Waiver path, but worth knowing for the right resident.

Part 8 — Staffing and the Care Team

Staffing is your biggest cost and your biggest challenge, and brain injury raises the bar: these residents need more supervision and more consistency than a typical assisted-living census, so plan ratios and awake overnight coverage around real acuity rather than a fixed minimum. Consistency matters as much as coverage — a rotating cast of unfamiliar faces is destabilizing for someone with memory and processing deficits — so invest in retention through fair pay, real training, respect, and a manageable workload, because turnover is expensive in both money and resident wellbeing.

Think of yourself as running a care team, not just a staff roster: the resident's therapists, physicians, behavioral providers, and waiver or MyCare case manager are all part of it, and your job is to coordinate them around each resident's plan and communicate changes quickly. Every direct-care staff member must also pass the criminal records check and be trained and competency-validated before working with residents — and, where they administer or assist with medications, trained specifically for that.

Part 9 — The Program and Accreditation

Beyond housing and personal care, a strong brain-injury RCF assesses each resident, builds an individualized plan with community-integration goals, uses brain-injury-informed cueing and behavioral supports, administers medications reliably, and plans community re-entry — tracking outcomes over time.

In practice that means a real assessment at admission of each resident's cognitive, physical, behavioral, and daily-living abilities, refreshed when things change; an individualized plan with concrete, measurable goals (managing a medication routine with fewer prompts, preparing a simple meal, using community transportation) developed with the resident and coordinated with their therapists and case manager; and a predictable daily structure — consistent routines, visual schedules, memory and organization supports — because structure is medicine for a brain injury. Staff use positive, least-restrictive behavioral supports and trained de-escalation rather than confrontation, and each resident's specific strategies are written down so care is the same on every shift.

Community integration is what sets a strong home apart: supervised outings, practicing real-world skills, and connecting residents to vocational rehabilitation where their goals include work. And because payers and accreditors increasingly ask whether your program actually works, track outcomes at the program level — functional gains, length of stay, where residents go when they leave — which becomes your proof of value when you talk to referral sources and payers.

The interdisciplinary approach is what turns a residence into a rehabilitation-oriented program: rather than each provider working in isolation, an interdisciplinary team — the resident and family, physician and specialists, nursing, therapists, the waiver or MyCare case manager, and trained direct-care staff — aligns the resident's therapy goals, behavioral plan, medication regimen, and daily routine so they reinforce one another. Treating the program as transitional, aimed at moving residents toward the most independent setting they can achieve rather than indefinite custodial care, is central both to good brain-injury care and to how payers evaluate you.

Accreditation — a market advantage. Ohio managed-care plans (including MyCare Ohio) and other payers often prefer national rehabilitation accreditation (the Joint Commission or CARF, which specializes in brain-injury programs) for brain-injury providers. Building your program to align with these frameworks — without reproducing their proprietary standards — strengthens payer contracting and survey readiness. Your Policy & Procedure

Manual is structured to support this alignment, and your outcome data (Part 10) is the evidence that makes an accreditation and payer case.

Part 10 — Timeline, Costs, and Two Worked Examples

Rough sequence

Plan for several months from start to first billable day: form the entity and prepare the home → screen and train staff → submit the RCF application (60 days ahead) with documents and fee → pass the licensure inspection → receive your license → certify as an Assisted Living Waiver provider and enroll in Medicaid → line up funding and referrals → begin admissions. Verify current timeframes with ODH and ODA.

Typical startup costs to budget

- The home (lease or purchase), accessibility modifications, furnishings, and fire-safety systems.
- Staff training, criminal records checks, and the medication-administration staffing your residents require.
- The ODH license fee (and the optional \$2,250 expedited-inspection fee) and local zoning/permit/occupancy costs.
- Insurance, business setup, and your Policy & Procedure Manual and Forms Packet.
- A cash reserve — plan for the home to run partly empty for a while; undercapitalization, not lack of demand, is what closes new homes.

Worked example A — a small RCF in the Columbus metro

A single-family or small residential building is licensed as an RCF serving adults with brain injury in the Columbus area. It takes referrals from central-Ohio rehabilitation and trauma programs — Ohio State's Wexner Medical Center and its Dodd Rehabilitation Hospital are primary sources — and from Assisted Living Waiver case managers at the regional PASSPORT Administrative Agency (the Central Ohio Area Agency on Aging). Its funding is a mix: residents approved for the Assisted Living Waiver or enrolled in MyCare Ohio (Franklin County is a MyCare county) have their services billed at the ODA tier rate, with room and board paid privately; other residents are private-pay. Because the waiver has limited slots, the home budgets a private-pay base and administers medications in-house, which is its edge over adult-care-facility competitors. Confirm current tier rates with ODA before projecting revenue.

Worked example B — an RCF in the Cincinnati metro

An RCF in the Cincinnati area runs the same model. It draws referrals from UC Health and its Daniel Drake Center for post-acute and rehabilitation care and from Hamilton County trauma and rehab programs, plus MyCare plans (Hamilton is a MyCare county) and the regional PAA (Council on Aging of Southwestern Ohio). The licensing, staffing, medication, and billing steps are identical to the Columbus home; the difference is the local market and the local permitting offices below. Given the waiver waitlist, it leans on private pay while adding waiver- and MyCare-funded residents as authorizations come through.

In both markets, discharge planners at rehabilitation hospitals and trauma centers, and waiver/MyCare case managers, are your primary referral sources, alongside families seeking specialized brain-injury residential care. Actual revenue depends on each resident's authorized services and your private-pay rate; confirm current figures with ODH, ODA, and the resident's payer.

Getting your first residents

Referrals come from relationships, and they take time to build — start before you have an empty bed to fill. Introduce yourself and your program to the discharge-planning and case-management teams at your area's rehabilitation hospitals and trauma centers, to the waiver case managers at your PASSPORT Administrative Agency, and to the MyCare plan care managers in your county. Be specific about what you offer that ordinary residential care and adult family homes do not: reliable medication administration, brain-injury-trained staff, structure and supervision, and community-integration goals. Families searching for specialized brain-injury care are

a second channel — a clear, honest description of your program and your admission process helps them find and choose you. Keep your outcome data from the first resident forward, because it becomes the proof of value that referral sources and payers respond to.

Part 11 — Common Mistakes That Delay or Sink an RCF

Most homes that stumble don't fail on the big things — they fail on avoidable ones. Learn these before they cost you.

- **Banking on the Ohio Home Care Waiver.** It excludes licensed facilities. Build your model on the Assisted Living Waiver, MyCare, and private pay.
- **Picking the wrong license.** An adult care facility can't administer medications or bill the Assisted Living Waiver. For brain injury, license as an RCF.
- **Signing a lease before confirming zoning.** Get written confirmation that an RCF is allowed at the exact address first.
- **Under-budgeting the pre-revenue runway.** You pay rent, staff, and setup for months before your first billable resident — and before waiver certification comes through.
- **Thin training and files.** Missing criminal records checks, undocumented training, and incomplete resident records are the deficiencies that most often delay a first license.
- **Admitting residents you can't safely serve.** A resident needing continuous skilled nursing exceeds RCF authority. Assess honestly and refer up when needed.
- **Weak abuse-reporting discipline.** Report immediately to ODH and the county — never delay for internal review.

Part 12 — Your First 90 Days Operating

Getting the license is the start line, not the finish. Your first three months set whether the home runs smoothly.

- **Document everything from day one.** Assessments, care plans, the medication administration record, incident reports, drills, and training — your first survey after opening looks at what you actually did.
- **Finish waiver certification and Medicaid enrollment.** Get your Assisted Living Waiver certification and Provider Agreement in place so funding can flow.
- **Lock in referral relationships.** Introduce yourself to discharge planners at rehab hospitals and trauma centers and to waiver/MyCare case managers — referrals come from relationships and take time to build.
- **Build your staff into a team.** Consistency across shifts is everything for brain-injury residents — keep each resident's strategies visible and address turnover fast.
- **Run your medication system tightly.** Reliable, on-time medication administration is your core clinical promise — audit it from the start.
- **Track your outcomes from the first resident.** Outcome data becomes your proof of value for payers, accreditation, and referral sources.

Part 13 — When They Say “Check Locally”: Who to Call & What to Ask

Some things are handled by the State (ODH, ODA/ODM), and some by your local city or county (zoning, permits, fire). Here is who to call and exactly what to ask. Verify current numbers before you rely on them.

The statewide offices (same for everyone)

For...	Who to contact
Licensing your RCF (application, standards, survey)	Ohio Department of Health — Bureau of Long-Term Care / Office of Health Assurance and Licensing (odh.ohio.gov); Complaint Unit 800-342-0553

For...	Who to contact
Assisted Living Waiver provider certification	Ohio Department of Aging — Provider Relations 614-645-7250 or 800-589-7277 (aging.ohio.gov certification)
Waiver enrollment & case management (your residents)	Your regional PASSPORT Administrative Agency — find it at 1-866-243-5678
Medicaid / waiver enrollment help	Ohio Benefits Consumer Helpline 1-844-640-6446; Ohio Benefits Long-Term Services & Supports (OBLTSS) 844-644-6582
MyCare Ohio	Through the member's MyCare plan care manager (Aetna, Buckeye, CareSource, Molina, UnitedHealthcare)
Resident advocacy	Ohio Long-Term Care Ombudsman — 1-800-282-1206 · elderrights@age.ohio.gov
Reporting abuse, neglect, or exploitation	911 if in immediate danger · Adult Protective Services 1-855-644-6277 (or your county JFS) · ODH 800-342-0553

The local calls — and exactly what to ask

- ① **City or County Zoning / Planning — “Can I legally run a residential care facility here?”**
 - “I want to open a residential care facility — a licensed home where adults with brain injury live and receive personal care, supervision, and medication administration. Is that an allowed use at [address]?”
 - “What local permits, occupancy, or zoning approvals apply? Can you confirm in writing?”
 - If they hesitate: homes for people with disabilities are protected as a residential use under the federal Fair Housing Act — ask them to point to the specific local rule.
- ② **Building / Occupancy — confirm your certificate of occupancy is in use group R-4, I-1, or I-2.**
- ③ **Fire — “What fire-safety inspection and systems does an RCF of [size] need here, and can you schedule the inspection I’ll need for my ODH license?”**

Your two-metro call list — fully worked

COLUMBUS (FRANKLIN COUNTY)

- **Zoning & permits:** City of Columbus Department of Building & Zoning Services, 111 N. Front St. — Customer Service 614-645-7433; Chief Zoning Official 614-645-2208; Engineering Plan Review 614-645-0032; columbus.gov/bzs. Apply online through the city's Citizen Access Portal.
- **Business/facility licenses:** handled through Building & Zoning Services and the Citizen Access Portal.
- **Fire:** City of Columbus Division of Fire, Fire Prevention Bureau (confirm the inspection required for your ODH license).
- **The local quirk:** metro Columbus is a patchwork — suburbs like Dublin (614-410-4670), Gahanna, Grandview Heights, Worthington, and Whitehall run their own building departments, so confirm which jurisdiction your exact address is in before you call.
- **Referrals:** Ohio State Wexner Medical Center / Dodd Rehabilitation Hospital discharge planning; the Central Ohio Area Agency on Aging (your PAA).

CINCINNATI (HAMILTON COUNTY)

- **One-stop Permit Center:** City of Cincinnati Department of Buildings & Inspections, Development & Permit Center, 805 Central Ave, Suite 500 — which combines Buildings, Fire, Health, Water, and Sewer in one place.
- **Zoning & ‘is my use allowed’:** Zoning/Variance 513-352-2430 (ZoningInfo@cincinnati-oh.gov); Small Business Development (confirms whether your use is permitted at a location) 513-352-4847.
- **Permits & inspections:** permit applications 513-352-3271; inspections 513-352-3275.

- **The local quirk:** Cincinnati has hillside overlays, historic districts (Historic Conservation review), and flood-plain rules; ask about a Coordinated Site Review, which gets you written feedback from every department early.
- **Referrals:** UC Health / Daniel Drake Center for post-acute & rehabilitation care; the Council on Aging of Southwestern Ohio (your PAA).

How to find your own city or county's numbers (if you're elsewhere)

1. Confirm your jurisdiction: search "[your city] OH building and zoning" or "[your county] OH building department." Many Ohio suburbs run their own building departments.
2. Ask Zoning the "allowed use" question above — and get the answer in writing.
3. Ask the Fire Marshal what inspection your RCF needs for the ODH license.
4. For funding, call the Ohio Department of Aging (800-589-7277) and find your PAA (1-866-243-5678).
5. Not sure who covers you? Call your county's main line and ask for the building/zoning department.

Core Rules & References

- **O.A.C. Chapter 3701-16** — Residential Care Facilities (the primary licensing rules; sections 3701-16-01 to 3701-16-18).
- **O.R.C. Chapter 3721** — Nursing Homes and Residential Care Facilities (residents' rights 3721.10–3721.17; RCF nursing services 3721.011; criminal records checks 3721.121; vaccinations 3721.041).
- **Assisted Living Waiver** — O.A.C. Chapter 5160-33; provider certification via the Ohio Department of Aging.
- **Ohio Home Care Waiver exclusion** — O.A.C. 5160-46-02(A)(7) (why it can't fund a licensed facility).
- **Rules & statutes online** — codes.ohio.gov; licensing forms at odh.ohio.gov.

Key Terms at a Glance

The alphabet soup you'll hear, in plain language:

- **RCF (Residential Care Facility):** Ohio's assisted-living license — the one that can administer medications and bill the Assisted Living Waiver.
- **ACF (Adult Care Facility):** Ohio's smaller adult family/group home license — cannot administer medications; not the fit for brain injury.
- **ODH:** Ohio Department of Health — licenses and surveys your RCF.
- **ODA / ODM:** Ohio Departments of Aging and Medicaid — run the Assisted Living Waiver that funds services in your RCF.
- **Assisted Living Waiver:** the Medicaid waiver that pays for services (not room and board) in an ODH-licensed RCF.
- **Ohio Home Care Waiver:** a Medicaid waiver that excludes licensed facilities — it will not fund your home.
- **MyCare Ohio:** managed care for people with both Medicare and Medicaid in 29 counties.
- **PAA (PASSPORT Administrative Agency):** your regional agency for waiver enrollment and case management.
- **NF level of care:** the nursing-facility level-of-care determination a resident must meet to qualify for the waiver.
- **CARF / Joint Commission:** the two national accreditors payers recognize; CARF specializes in brain-injury and rehabilitation programs.

Your Compliance Foundation

Opening the home is the first milestone; staying compliant with O.A.C. 3701-16 — and ready for accreditation — is the ongoing work. Two companion resources are built for that: a Policy & Procedure Manual covering the licensure

standards and the accreditation framework, and a Forms Packet containing the forms your policies reference. Together they give you the documentation backbone ODH and an accreditor expect.

Before you rely on any step in this guide

Verify current requirements, forms, fees, and rates with ODH and, for funding, with the Ohio Department of Aging, your PASSPORT Administrative Agency, and the resident's payer. Remember that the Ohio Home Care Waiver will not fund a licensed facility — confirm each resident's funding before admission.